RIVERSIDE GENERAL HOSPITAL

Department of Internal Medicine

COMPREHENSIVE LABORATORY AND CLINICAL REPORT

PATIENT AND SPECIMEN DETAILS

Patient name:	Meera R. Sharma	**Medical record no.:**	MRN-40918-227
Age / sex:	54 years / Female	**Date of birth:**	1971-03-18
Ward / bed:	General Medicine / 4-West, Bed 12	**Attending physician:**	Dr. Anil K. Verma, MD (Internal Medicine)
Referring clinician:	Dr. Sneha Iyer, MBBS (City Family Clinic)	**Specimen collected:**	2026-02-11, 07:15
Report issued:	2026-02-12, 16:40		

VITAL SIGNS ON ADMISSION

Parameter	Value	Unit	Reference	Flag
Blood pressure	148/92	mmHg	< 130/85	HIGH
Heart rate	88	bpm	60-100	normal
Respiratory rate	18	/min	12-20	normal
Temperature	37.1	deg C	36.1-37.2	normal
SpO2 (room air)	97	%	95-100	normal
Height / weight	158 / 74.5	cm / kg	-	BMI 29.8

LABORATORY RESULTS

Parameter	Value	Unit	Reference	Flag
Haemoglobin	11.2	g/dL	12.0-15.5	LOW
White blood cells	9,400	/uL	4,000-11,000	normal
Platelet count	2.62	lakh/uL	1.5-4.5	normal
Fasting plasma glucose	138	mg/dL	70-100	HIGH
HbA1c	7.4	%	< 5.7	HIGH
Total cholesterol	214	mg/dL	< 200	HIGH
LDL cholesterol	141	mg/dL	< 100	HIGH
HDL cholesterol	44	mg/dL	> 50	LOW
Triglycerides	186	mg/dL	< 150	HIGH
Serum creatinine	0.9	mg/dL	0.6-1.1	normal
Blood urea nitrogen	16	mg/dL	7-20	normal
AST (SGOT)	31	U/L	10-40	normal
ALT (SGPT)	38	U/L	7-56	normal
TSH	2.8	uIU/mL	0.4-4.0	normal

Flags: HIGH above the reference range, LOW below the reference range.

CHIEF COMPLAINT

54-year-old woman with a known history of type 2 diabetes mellitus presents with fatigue, increased thirst and intermittent blurred vision over the past six weeks. No chest pain, no shortness of breath, no fever and no weight loss reported.

HISTORY OF PRESENT ILLNESS

Diagnosed with type 2 diabetes mellitus eight years ago and on metformin since. Hypertension diagnosed three years ago, currently treated with amlodipine. Family history of ischaemic heart disease in a first-degree relative. Non-smoker, occasional social alcohol. No known drug allergies. Recent home glucose readings have been running high before meals.

ASSESSMENT AND PLAN

1. Type 2 diabetes mellitus, poorly controlled.
HbA1c of 7.4% is above the individualised target of below 7.0%, and fasting plasma glucose is 138 mg/dL. The blurred vision is consistent with osmotic lens swelling from hyperglycaemia and is expected to improve as glucose control improves.

2. Essential hypertension, uncontrolled.
Blood pressure of 148/92 mmHg is above the target of below 130/85 mmHg for a patient with diabetes. Home monitoring twice daily has been advised.

3. Mixed dyslipidaemia.
LDL cholesterol of 141 mg/dL and triglycerides of 186 mg/dL raise cardiovascular risk. Statin therapy has been discussed and is offered.

4. Overweight (BMI 29.8).
Dietary counselling and a graded exercise plan of 30 minutes of brisk walking five days a week have been arranged.

MEDICATIONS AT DISCHARGE

Metformin 500 mg - one tablet twice daily after meals.

Amlodipine 5 mg - one tablet once daily in the morning.

Atorvastatin 20 mg - one tablet at bedtime (new, to be discussed at review).

Aspirin 75 mg - once daily after food (to be reviewed at follow-up).

FOLLOW-UP INSTRUCTIONS

1. Review in the outpatient clinic after four weeks with a repeat HbA1c.
2. Self-monitor fasting and post-meal glucose daily; bring the logbook.
3. Dietary advice from the hospital dietitian, appointment already booked.
4. Return earlier if there is chest pain, breathlessness, dizziness or fainting.

CLOSING NOTE

This report is a fictional sample produced for a software demonstration. The patient, hospital, clinicians, values and dates are all invented.

Dr. Anil K. Verma, MD (Internal Medicine)
Consultant Physician, Riverside General Hospital
Electronically signed: 2026-02-12 16:40 IST